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Project #39: Appropriate Preoperative Screening in Low-Risk Surgeries

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BACKGROUND

Research shows that routine preoperative testing in elective low risk surgeries:

- Costs the U.S. an estimated \$85.2 million annually.
- May add to patient's personal financial burden.
- Does not improve patient outcomes.
- Clinically insignificant results can lead to delays in needed care prolonging pain and suffering and additional decompensation.

Clinically insignificant testing can also lead to a cascade of additional unnecessary testing. (1)

AIM/PURPOSE

PROBLEM STATEMENT:

- Unnecessary testing leads to increased cost and potential delays in care with no ROI (Return on Investment).

GOAL:

- Decrease unnecessary preoperative testing rates in patients undergoing any of 3 elective low risk surgeries (lap cholecystectomy, minor hernia repair, and breast lumpectomy) by 20% by the end of 2023.

PLANNING

JANUARY 2023:

- Begin a pilot study of 3 elective low risk surgeries to gain insight into our practices at HFH. This will determine our baseline.
- Review and analyze quarterly data from MVC (Michigan Value Collaborative)
- Develop a multidisciplinary team to compare current practice to best practice.
- Develop a more robust policy/protocol to determine who does/does not require preoperative testing.
- Create Decisions aids to assist in determining risk score and appropriateness.
- Update Epic to reflect new protocols
- Educate & roll-out new protocols
- Continue data collection and PDCA cycles

IMPLEMENTATION

- A PAT (Pre-Anesthesia Testing) Risk Assessment Questionnaire and an updated Lab Testing Grid were developed and implemented 6/28/2023.
- The PAT Standing Order set and the Nursing Implemented Standing Orders Policy were revised to reflect the new clinical decision support (CDS) tools.
- New protocol was communicated to surgeons and the PAT nursing staff in April and July.
- Questionnaire and Order Sets were updated in EMR (electronic medical record) by the Epic orders team.

PROJECT BASELINE RATE

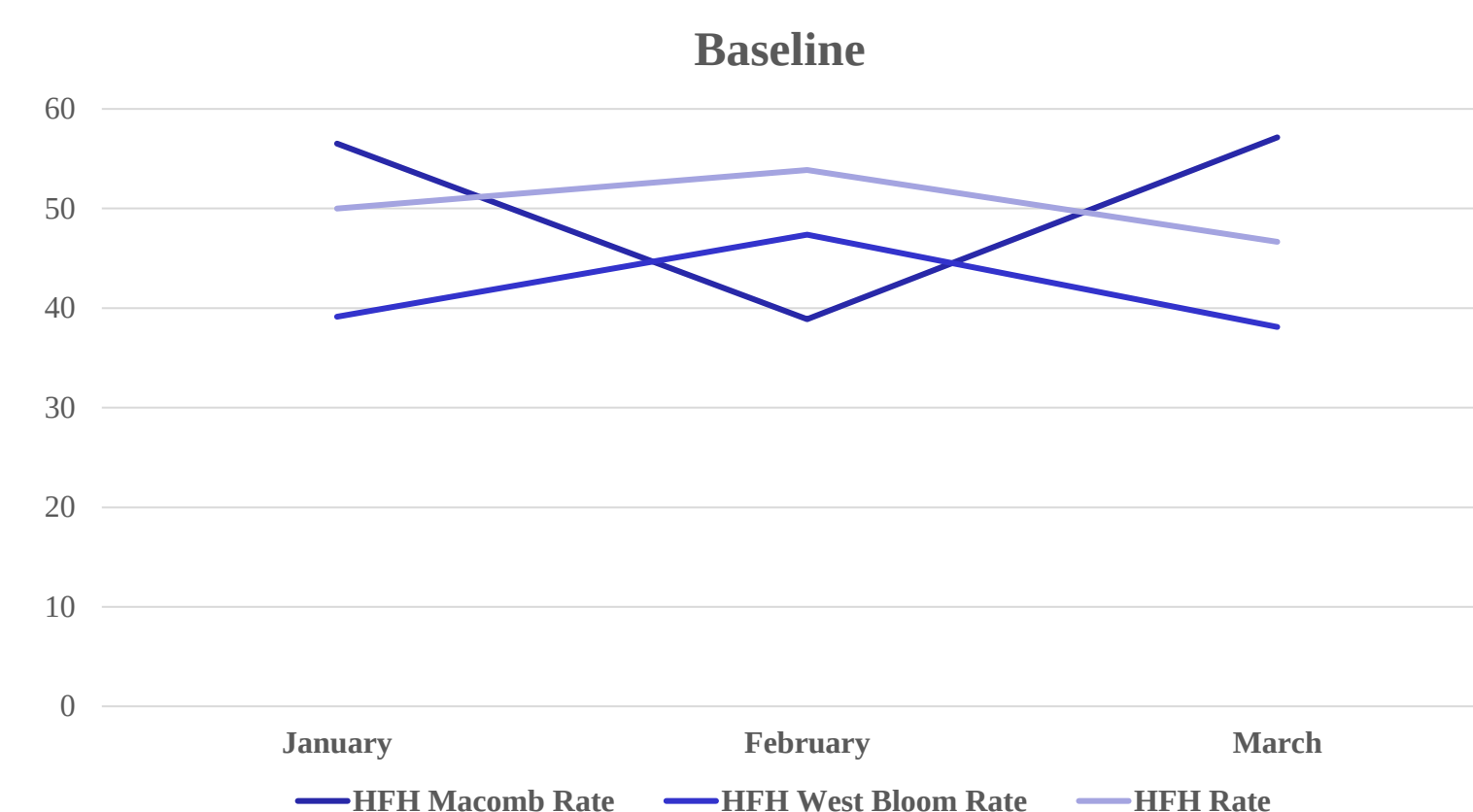


Figure 1. Baseline Rate

PROGRESS FOLLOWING INTERVENTION

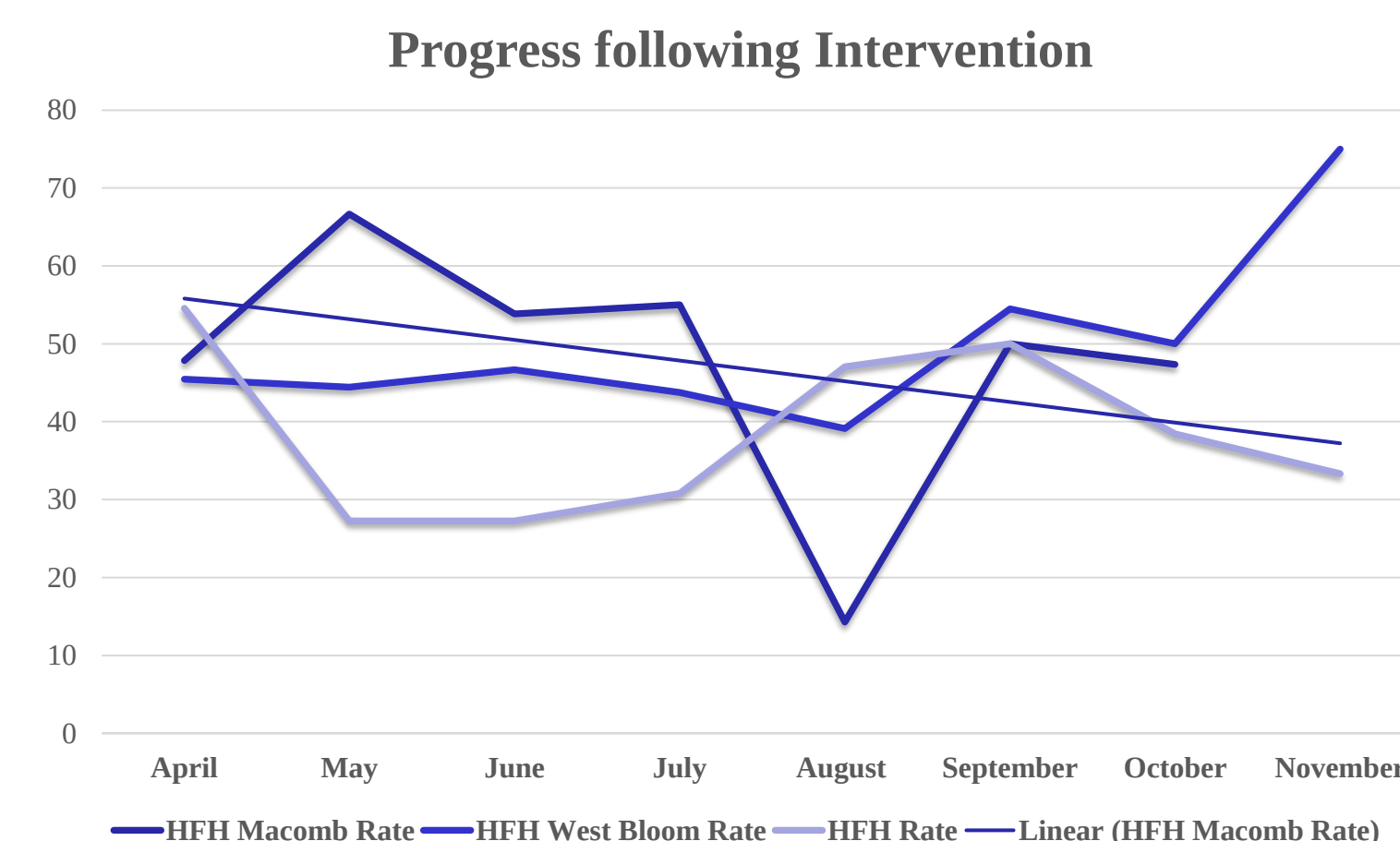


Figure 2. Progress following project intervention.

BARRIERS

- Private physicians who order their own labs.
- Patients who follow with other health systems.
- Labs ordered by other providers not related to surgery.
- Oncology patients undergoing chemotherapy require ongoing lab work.
- The amount and timing of approvals to get EMR updated.
- Lack of awareness in the community regarding existing guidelines and best practices.

Sources of Unnecessary Preop Testing

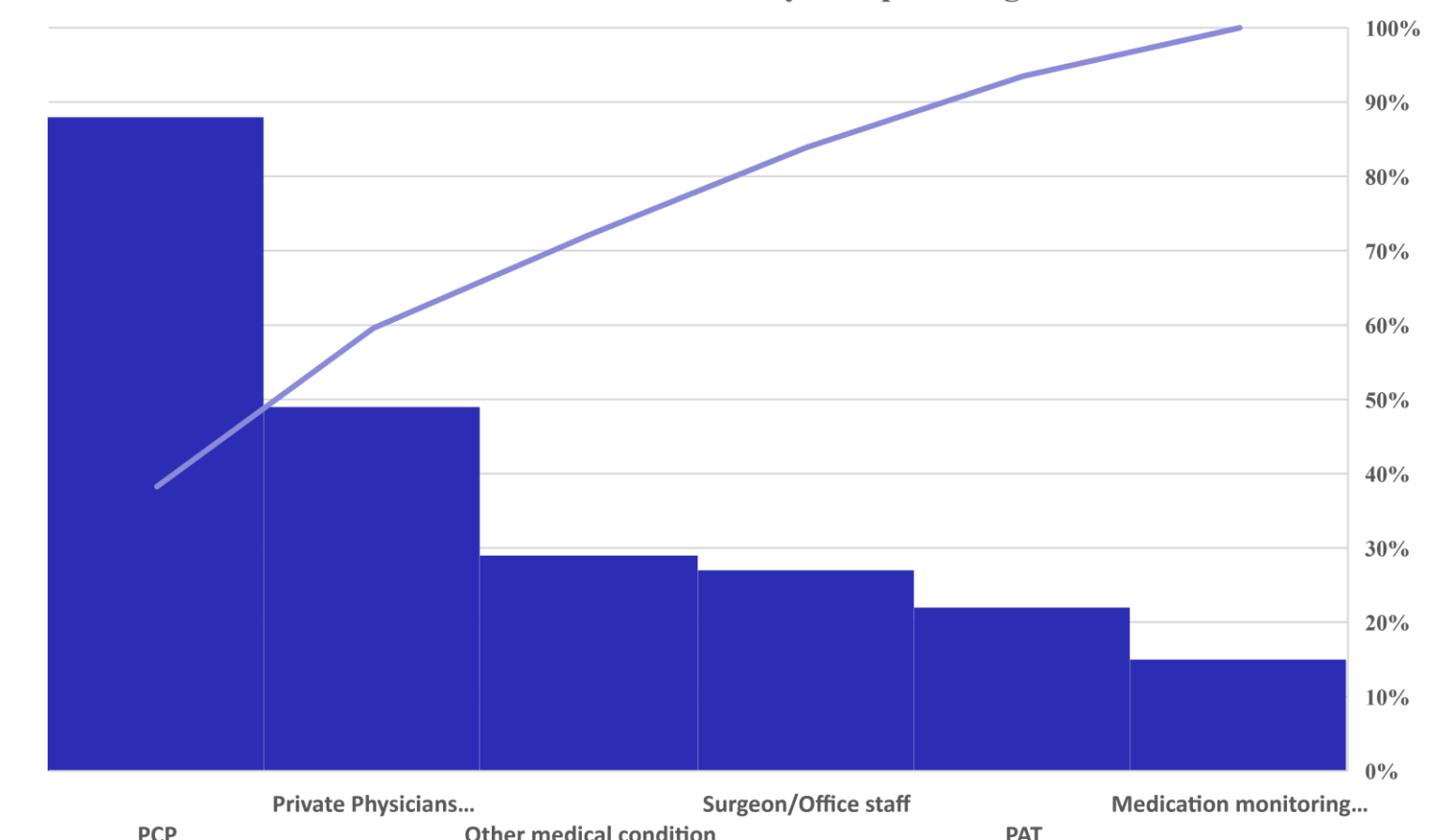


Figure 3. Pareto Chart demonstrating sources of unnecessary preop testing

NEXT STEPS

- Continue PDCA cycles throughout 2024 with focus on what we can control.
- Share results of 2023 work at multidisciplinary meeting and with senior leadership.
- Provide timely feedback to individuals regarding orders outside of protocol.
- Assess triggers for implementation of pre-operative clearance by providers.
- Communicate new protocol to Primary Care Physician groups.

REFERENCES

1. MVC (Michigan Value Collaborative) workgroup presentation. Hari Nathan, MD, PhD Director, Michigan Value Collaborative March 2023